

Obs & Gynae Ward Quick Reference

A high-yield cheat-sheet for the Ugandan labour ward and obstetric wards, covering drug doses (MgSO₄, antihypertensives, uterotonics, steroids, misoprostol, antibiotics, anti-D, ANC supplements), scoring systems, danger signs, normal values in pregnancy, key definitions and partograph rules.

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NOTE

This page is a memory aid for the ward. Always check doses against the **Uganda Clinical Guidelines 2023**, your unit protocol and a senior before prescribing. Doses are for adults with normal renal function unless stated.

1. Magnesium sulphate (MgSO₄)

Strength check: 50% MgSO₄ = **0.5 g per mL** → 10 mL = **5 g**. To make **20%** for IV use: **8 mL of 50% (4 g) + 12 mL water for injection = 20 mL**.

Regimen	Loading dose	Maintenance	Duration
Pritchard (IM), the Uganda standard	4 g of 20% IV over 5–10 min + 10 g of 50% IM (5 g deep IM into each buttock, each with 1 mL of 2% lignocaine) = 14 g	5 g of 50% IM every 4 h , alternate buttocks (+ 1 mL 2% lignocaine)	24 h after delivery or last fit (whichever is later)
Zuspan (IV)	4 g IV over 5–20 min	1 g/h IV infusion	24 h after delivery or last fit
Recurrent fit	2 g of 20% IV over 5 min	Continue regimen	Think of another cause if fits continue
Fetal neuroprotection (<32 weeks)	4 g IV over 20–30 min	1 g/h	Until birth or max 24 h
At HC II/III before referral	Full loading dose (IM 10 g ± IV 4 g)	Send the next 5 g IM dose with the patient	Due 4 h after loading

DRUG DOSING

Before every dose (hourly on infusion): "R-R-U"

- Respiratory rate $\geq 16/\text{min}$
- Reflexes (patellar) **present**
- Urine output $\geq 30 \text{ mL/h}$ ($\geq 100 \text{ mL}$ in 4 h)

If RR < 16 or reflexes absent $\rightarrow$ **withhold**. If urine $< 30 \text{ mL/h}$ $\rightarrow$ withhold/halve. Respiratory arrest $\rightarrow$ ventilate + **calcium gluconate 1 g (10 mL of 10%) IV over 10 min**.

Serum Mg^{2+}	Effect
2–3.5 mmol/L (4.8–8.4 mg/dL)	Therapeutic
4–5 mmol/L (~ 10 –12 mg/dL)	Loss of patellar reflexes
6–7.5 mmol/L (~ 15 –17 mg/dL)	Respiratory depression
$> 12 \text{ mmol/L}$ ($> 30 \text{ mg/dL}$)	Cardiac arrest

If no MgSO_4 : diazepam 10 mg IV slowly over 2 min (repeat once if fits recur); rectal 20 mg if no IV access.

2. Antihypertensives in pregnancy

Treat urgently if BP $\geq 160/110$ (within 30–60 min). Target **140–150/90–100** acutely; **$\sim 135/85$** for maintenance. Do not drop diastolic below 80.

Acute (severe hypertension)

Drug	Dose	Route	Repeat	Max / notes
Nifedipine IR	10 mg	Oral (swallow, not sublingual)	After 20–30 min if still $\geq 160/110$ (10–20 mg)	~ 30 –40 mg in the first hour. Headache, flushing, tachycardia
Hydralazine	5 mg (5–10 mg)	IV slowly over 2–5 min	Every 20–30 min	20 mg IV per episode (WHO: or 12.5 mg IM 2-hourly). Tachycardia, headache, hypotension $\rightarrow$ fetal distress
Labetalol	20 mg	IV over 2 min	Then 40, 80, 80 mg every 10 min	300 mg total. Avoid in asthma. Rarely available in Uganda

Maintenance (oral)

Drug	Dose	Frequency	Notes
Methyldopa	250–750 mg	8-hourly (max 3 g/day)	Slow onset; not for acute use ; switch postpartum (depression)
Nifedipine MR (retard)	20–40 mg	12-hourly (max 120 mg/day)	Common in Uganda
Labetalol	100–400 mg	8–12-hourly (max 2.4 g/day)	Avoid in asthma
Amlodipine	5–10 mg	Once daily	Also good postpartum
Enalapril (postpartum only)	5–20 mg	Once–twice daily	Contraindicated antenatally ; safe in breastfeeding

RED FLAG

Avoid in pregnancy: ACE inhibitors, ARBs (fetal renal damage), atenolol (FGR), diuretics (except pulmonary oedema). **Never give ergometrine** to a hypertensive woman.

3. Uterotonics and PPH drugs

Drug	Prevention (AMTSL)	Treatment of PPH	Contraindications / notes
Oxytocin (first line)	10 IU IM (or IV slowly) within 1 min of birth	10 IU IV slowly + infusion 20–40 IU in 1 L NS/RL (e.g. 20 IU in 1 L at 60 drops/min, then 40 drops/min)	Store cool (2–8°C). Max about 3 L of oxytocin-containing fluid (water intoxication)
Carbetocin (heat-stable)	100 mcg IM or slow IV single dose	Not for treatment	Heat-stable; WHO-recommended alternative for prevention
Ergometrine	0.2 mg IM	0.2 mg IM or slow IV ; repeat 0.2 mg IM after 15 min, then 4-hourly if needed	Max 5 doses (1 mg)/24 h. CI: hypertension, pre-eclampsia, heart disease. Refrigerate
Misoprostol	600 mcg orally (where oxytocin is unavailable; community distribution)	800 mcg sublingually	Shivering and fever common. Heat-stable
Tranexamic acid (TXA)	Not routine	1 g IV over 10 min within 3 h of birth ; second 1 g if bleeding continues after 30 min or restarts within 24 h	WOMAN trial. Useless after 3 h. For any PPH cause
Carboprost (15-methyl PGF _{2α})		250 mcg IM every 15 min, max 8 doses (2 mg)	CI asthma. Rarely available
Calcium gluconate		1 g IV over 10 min	For MgSO ₄ toxicity (and massive transfusion)

UGANDA CONTEXT

WHO 2023 / E-MOTIVE bundle for PPH: measure blood loss with a **calibrated drape**; trigger at **≥500 mL** (WHO 2023), or at **≥300 mL + any abnormal vital sign** (WHO/FIGO/ICM 2025 consolidated guideline); then immediately and together: **Early detection**, **Massage the uterus**, **Oxytocic drugs**, **Tranexamic acid**, **IV fluids**, **Examination** (genital tract, placenta) and **Escalation**.

4. Antenatal corticosteroids

Drug	Dose	Route	Frequency	Total
Dexamethasone (first choice in Uganda)	6 mg	IM	12-hourly	4 doses (24 mg)
Betamethasone	12 mg	IM	24-hourly	2 doses (24 mg)

WHO 2022 conditions: 24–34 weeks (Uganda: usually 28–34, lower at NICU centres); **accurate GA**; birth likely **within 7 days**; **no maternal infection**; **adequate childbirth care**; **adequate newborn care** (resuscitation, thermal care, KMC, feeding, infection treatment, oxygen/CPAP). One repeat course allowed if **≥7 days** since the first and still at high risk. Not routine **≥34 weeks**.

Tocolysis (only to buy 48 h for steroids/transfer): nifedipine 20 mg orally, then 10–20 mg every 6–8 h for up to 48 h. Not with chorioamnionitis, APH, fetal distress/death, severe PE, or ≥ 34 weeks. Avoid betamimetics (salbutamol).

5. Misoprostol regimens by indication

Indication	Gest ation	Regimen
Incomplete miscarriage	<13 weeks	600 mcg orally once, or 400 mcg sublingually once
Missed miscarriage (early fetal demise)	<14 weeks	WHO 2022 preferred: mifepristone 200 mg orally, then misoprostol 800 mcg vaginally/sublingually 1–2 days later. Misoprostol alone (FIGO): 800 mcg vaginally (or 600 mcg sublingually); repeat after 3 h if needed (max 2 doses)
Cervical priming before MVA / surgical evacuation	<13 weeks	400 mcg sublingually 1 h or vaginally 3 h before the procedure
Termination (legal indication) / incomplete abortion with a uterus ≥ 13 –14 weeks	13–24 weeks	400 mcg vaginally, sublingually or buccally every 3 h until expulsion (WHO 2022)
Fetal death	14–28 weeks	400 mcg vaginally/sublingually every 4–6 h (WHO 2022, ideally after mifepristone 200 mg) or 200 mcg every 4–6 h (FIGO). Use the lower dose nearer 28 weeks (FIGO: 100 mcg every 4 h at 27–28 weeks) and with a uterine scar
Fetal death	>28 weeks	25 mcg vaginally every 6 h or 25 mcg orally every 2 h (FIGO). WHO MCPC: if no response after 2 doses, 50 mcg every 6 h; never more than 50 mcg per dose
Induction of labour, live fetus at term	≥ 37 weeks	25 mcg vaginally every 6 h or 25 mcg orally every 2 h (oral solution)
PPH prevention	After birth	600 mcg orally
PPH treatment	After birth	800 mcg sublingually

RED FLAG

Previous caesarean section / uterine scar: avoid misoprostol for induction in the third trimester (uterine rupture). Lower doses and extreme caution in the second trimester. Always stop if there is **tachysystole** (>5 contractions in 10 min) or fetal distress.

6. Antibiotic regimens common in obstetrics

Indication	Regimen	Duration
Puerperal sepsis / maternal sepsis / septic abortion	Ampicillin 2 g IV 6-hourly + gentamicin 5 mg/kg IV once daily + metronidazole 500 mg IV 8-hourly (alternative: ceftriaxone 1–2 g IV daily + metronidazole 500 mg IV 8-hourly)	IV until afebrile for 48 h (WHO MCPC), then oral (e.g. amoxicillin + metronidazole) to complete about 7 days in total; longer if an abscess or complicated infection, per local protocol. Evacuate retained products; drain pus
PPROM	Erythromycin 250 mg orally 6-hourly	10 days or until labour. Avoid co-amoxiclav (NEC)
Intrapartum GBS prophylaxis (preterm labour, ROM >18 h, fever, previous GBS baby)	Benzylpenicillin 3 g (5 MU) IV, then 1.5–1.8 g (2.5–3 MU) IV 4-hourly; or ampicillin 2 g IV then 1 g 4-hourly; allergy: clindamycin 900 mg IV 8-hourly	Until delivery
Chorioamnionitis	Ampicillin 2 g IV 6-hourly + gentamicin 5 mg/kg IV daily (+ metronidazole 500 mg IV 8-hourly if CS)	Deliver. Stop after vaginal birth if well; after CS continue until 48 h afebrile
Caesarean section prophylaxis	Cefazolin 2 g IV (3 g if >120 kg) or ampicillin 2 g IV , single dose 15–60 min before skin incision (Uganda often uses ceftriaxone 1–2 g IV)	Single dose (repeat if >4 h surgery or blood loss >1.5 L)
Pyelonephritis in pregnancy	Admit; ceftriaxone 1–2 g IV daily (or ampicillin + gentamicin)	Until afebrile 24–48 h, then oral to complete 10–14 days ; urine culture; repeat culture after treatment
Asymptomatic bacteriuria / cystitis	Nitrofurantoin 100 mg 12-hourly (avoid at term) or amoxicillin 500 mg 8-hourly (if sensitive), or cefalexin 500 mg 8–12-hourly	5–7 days
Syphilis (early)	Benzathine penicillin 2.4 MU IM single dose (late/unknown duration: weekly × 3)	Treat partner too
Manual removal of placenta / 3rd–4th degree tear	Single dose ampicillin 2 g IV (or cefazolin) ± metronidazole 500 mg IV	Single dose

7. Anti-D immunoglobulin (Rh-negative, non-sensitised women)

Situation	Dose	Timing
After delivery of an Rh-positive baby	Uganda (usual vial): 1,500 IU (300 mcg) IM. UK: 500 IU IM + Kleihauer to guide extra doses	Within 72 hours (some benefit up to 10 days)
Routine antenatal prophylaxis (where available)	1,500 IU (300 mcg) IM at 28 weeks (single-dose regimen), or 500 IU at 28 and 34 weeks (UK two-dose regimen)	
Sensitising event before 20 weeks (surgical evacuation, ectopic, molar pregnancy, miscarriage ≥12 weeks, APH, trauma)	UK: 250 IU (50 mcg) IM (before 12 weeks only after surgical evacuation, ectopic, molar pregnancy or heavy/repeated bleeding). Uganda: the vial usually stocked is 1,500 IU (300 mcg) IM , which is also acceptable	Within 72 h
Sensitising event 20 weeks or more (APH, ECV, abdominal trauma, IUFD, amniocentesis)	UK: 500 IU IM + Kleihauer to guide extra doses. Uganda (usual vial): 1,500 IU (300 mcg) IM	Within 72 h

UGANDA CONTEXT

Anti-D is expensive and not always stocked in public facilities. Check the blood group at booking, counsel Rh-negative women early, and **plan purchase before delivery**. Check the baby's cord blood group.

8. ANC supplements and prevention (Uganda)

Intervention	Regimen	Notes
Iron + folic acid	Ferrous sulphate 200 mg (60 mg elemental) + folic acid 400 mcg once daily throughout pregnancy and 3 months postpartum	Anaemia treatment: 120 mg elemental daily . Take apart from calcium and tea
Folic acid (preconception)	400 mcg daily (5 mg if previous NTD, epilepsy on drugs, sickle cell)	Start 3 months before conception. Avoid 5 mg folic acid with SP (antagonises SP)
IPTp-SP	3 tablets of SP (sulfadoxine 500 mg/pyrimethamine 25 mg each) as directly observed therapy	From 13 weeks , then monthly (at least 4 weeks apart) until delivery; aim ≥ 3 doses. Not in the first trimester. Not with cotrimoxazole (HIV+)
Mebendazole	500 mg single dose	Second trimester
Calcium	1.5–2 g elemental daily from 20 weeks	Pre-eclampsia prevention (low-calcium diets)
Aspirin	75–150 mg at night from 12 to 36 weeks	High risk of pre-eclampsia
ITN	Sleep under a treated net every night	

Td (tetanus-diphtheria) schedule for women of childbearing age

Dose	When	Protection
Td1	At first contact / first ANC visit	None
Td2	At least 4 weeks after Td1	~1–3 years
Td3	At least 6 months after Td2	~5 years
Td4	At least 1 year after Td3	~10 years
Td5	At least 1 year after Td4	Through the childbearing years (probably longer)

Give Td2 at least 2 weeks before delivery for newborn protection against neonatal tetanus.

9. Bishop score

Score	0	1	2	3
Dilatation (cm)	Closed	1–2	3–4	≥ 5
Effacement (%) or cervical length	0–30% (>3 cm)	40–50% (2 cm)	60–70% (1 cm)	$\geq 80\%$ (0 cm)
Station (relative to ischial spines)	–3	–2	–1, 0	+1, +2
Consistency	Firm	Medium	Soft	
Position	Posterior	Mid	Anterior	

Interpretation: ≥ 8 favourable (ARM + oxytocin likely to succeed); ≤ 6 unfavourable (needs cervical ripening: misoprostol or Foley balloon).

10. APGAR score

Sign	0	1	2
Appearance (colour)	Blue or pale all over	Body pink, extremities blue	Pink all over
Pulse (heart rate)	Absent	<100/min	≥ 100 /min
Grimace (reflex irritability)	No response	Grimace	Cry, cough, sneeze
Activity (tone)	Limp	Some flexion	Active movement
Respiration	Absent	Slow, irregular, weak cry	Good, strong cry

Score at **1 and 5 minutes** (and 10 if <7). **7–10** normal; **4–6** moderately depressed; **0–3** severely depressed. **Do not wait for the APGAR to start resuscitation:** follow Helping Babies Breathe (dry, stimulate, clear airway if needed, **ventilate within the Golden Minute** if not breathing).

11. Glasgow Coma Scale

Eye opening (E)	Verbal (V)	Motor (M)
4 Spontaneous	5 Oriented	6 Obeys commands
3 To voice	4 Confused	5 Localises pain
2 To pain	3 Inappropriate words	4 Withdraws from pain
1 None	2 Incomprehensible sounds	3 Abnormal flexion (decorticate)
	1 None	2 Extension (decerebrate)
		1 None

Total 3–15. ≤ 8 = **protect the airway** (intubate/recovery position, call anaesthetist).

12. Shock index and haemorrhage recognition

Shock index = heart rate $\div$ systolic BP.

Obstetric shock index	Meaning
0.7–0.9	Normal in pregnancy/postpartum
≥ 0.9	Abnormal: significant blood loss likely; escalate
≥ 1.4	Needs urgent intervention (resuscitation, blood)
≥ 1.7	High risk of adverse outcome (critical)

CLINICAL PEARL

Pregnant women compensate well: **BP falls late** (after ~30% blood volume loss). Tachycardia, a rising shock index, pallor, cold peripheries, anxiety and reduced urine output come first. **Rule of 30:** SBP fall of 30 mmHg, HR rise of 30, RR >30, Hb/haematocrit fall of 30%, urine <30 mL/h $\rightarrow$ about 30% blood volume lost (moderate shock).

13. Obstetric danger signs (teach every mother)

During pregnancy	During labour	After delivery (mother)	Newborn
Vaginal bleeding	Labour >12 hours	Heavy bleeding (soaking a pad in <5 min)	Not feeding / unable to suck
Severe headache, blurred vision	Bleeding	Fever , foul-smelling lochia	Convulsions
Convulsions / fits	Leaking green/brown liquor	Severe abdominal pain	Fast breathing (≥ 60 /min), chest indrawing
Fever , very weak	Cord or hand coming out first	Convulsions, severe headache	Hot ($\geq 37.5^{\circ}\text{C}$) or cold ($< 35.5^{\circ}\text{C}$)
Severe abdominal pain	Fits, severe headache	Breathlessness, calf pain/swelling	Yellow palms/soles, or jaundice in the first 24 h
Fast or difficult breathing	Fever	Painful swollen breasts, perineal wound pus	Umbilical redness/pus
Reduced or absent fetal movements		Feeling very sad, thoughts of harm	Lethargy, floppy
Leaking liquor (water breaking)			
Swelling of face and hands			

14. Normal laboratory values in pregnancy

Test	Normal in pregnancy	Comment
Haemoglobin	≥ 11 g/dL (≥ 10.5 in 2nd trimester)	Anaemia < 11 ; moderate 7–9.9; severe < 7 (WHO)
White cell count	$6\text{--}16 \times 10^9/\text{L}$ (up to $\sim 25\text{--}30$ in labour/early puerperium)	Steroids raise WBC for 72 h
Platelets	$150\text{--}400 \times 10^9/\text{L}$ (gestational thrombocytopenia usually > 100)	< 100 = think HELLP/ITP/DIC
Creatinine	$35\text{--}70$ $\mu\text{mol/L}$ (0.4–0.8 mg/dL)	$> \sim 77\text{--}80$ $\mu\text{mol/L}$ is suspicious in pregnancy; > 90 $\mu\text{mol/L}$ is clearly abnormal
Urea	2–4.5 mmol/L	Falls because GFR rises $\sim 50\%$
Uric acid	Falls in early pregnancy, rises near term	High in pre-eclampsia
ALT / AST	Unchanged or slightly lower ($< \sim 35$ IU/L)	Raised = abnormal (HELLP, AFLP, hepatitis)
Alkaline phosphatase	Up to 2–4$\times$ normal	Placental origin: do not over-interpret
Albumin	Falls ($\sim 28\text{--}37$ g/L)	Haemodilution
Fibrinogen	Rises to 4–6 g/L	< 2 g/L in PPH = severe coagulopathy
PT / APTT	Unchanged or slightly shorter	
Fasting glucose	< 5.1 mmol/L	GDM (WHO 2013, 75 g OGTT): fasting ≥ 5.1 , 1 h ≥ 10.0 , 2 h ≥ 8.5
TSH	Lower in 1st trimester ($\sim 0.1\text{--}2.5$ mU/L)	Use trimester-specific ranges

Test	Normal in pregnancy	Comment
PaCO ₂	Lower (~3.7–4.2 kPa / 28–32 mmHg)	Respiratory alkalosis compensated by lower bicarbonate (18–22 mmol/L)
ESR, D-dimer, CRP	Raised	Poor diagnostic tests in pregnancy
Proteinuria	<300 mg/24 h; PCR <30 mg/mmol	Above = significant
BP	Falls in 2nd trimester, back to baseline by term	≥140/90 hypertension; ≥160/110 severe
Heart rate	Rises by 10–15 bpm	>100 at rest: look for a cause

15. Fetal skull diameters

Presentation	Diameter (engaging)	Length
Vertex, well flexed	Suboccipitobregmatic	9.5 cm
Vertex, partly flexed	Suboccipitofrontal	10.0 cm
Vertex, deflexed/OP	Occipitofrontal	11.5 cm
Brow	Mentovertical	13.5 cm <- too big: CS
Face (mentoanterior)	Submentobregmatic	9.5 cm
Face (partly extended)	Submentovertical	11.5 cm
Transverse: Biparietal 9.5 cm Bitemporal 8.0 cm		
Shoulders: Bisacromial 12 cm (reduces to ~9.5 cm with compression)		

Landmarks: anterior fontanelle (bregma) = diamond, 4 sutures; posterior fontanelle (lambda) = triangle (Y), 3 sutures. **Denominator:** vertex = occiput; face = mentum (chin); breech = sacrum; shoulder = acromion.

16. Key definitions

Term	Definition
Abortion / miscarriage	Pregnancy loss before 28 weeks in Uganda (WHO: <22 weeks or <500 g)
Stillbirth	Baby born with no signs of life at ≥28 weeks (WHO international comparison); Uganda MPDSR uses ≥28 weeks
Preterm birth	Before 37+0 weeks (extremely <28, very 28–<32, moderate/late 32–<37)
Term	37+0 to 41+6 weeks (early term 37–38+6; full term 39–40+6; late term 41–41+6)
Post-term	≥42+0 weeks
Low birth weight	<2,500 g (VLBW <1,500 g; ELBW <1,000 g)
PROM / PPROM	Rupture of membranes before labour at ≥37 / <37 weeks
Prolonged rupture of membranes	>18 hours (UK >24 h)
Primary PPH	Blood loss ≥500 mL within 24 h of birth (any route); severe ≥1,000 mL ; or any loss causing haemodynamic compromise. WHO/FIGO/ICM 2025: also diagnose at ≥300 mL plus any abnormal vital sign

Term	Definition
Secondary PPH	Abnormal bleeding from 24 h to 12 weeks postpartum (WHO: up to 6 weeks)
Hypertension in pregnancy	≥140/90 on two occasions; severe ≥160/110
Anaemia in pregnancy	Hb <11 g/dL; severe <7 g/dL
Fever	≥38.0°C
Prolonged labour	Active phase >12 h (Uganda) or crossing the action line; prolonged second stage: WHO 2018 / Labour Care Guide ≥3 h nulliparous, ≥2 h parous ; NICE/RCOG-style (and older ACOG) teaching, common on Ugandan wards, uses >2 h primigravida (>3 h with epidural) and >1 h multipara (>2 h with epidural)
Puerperium	6 weeks after delivery
Maternal death	Death during pregnancy or within 42 days of its end, from any cause related to or aggravated by pregnancy (not accidental). Late maternal death: 42 days to 1 year
Perinatal death	Stillbirth + early neonatal death (first 7 days)
Neonatal period	First 28 days of life
Maternal mortality ratio	Maternal deaths per 100,000 live births (Uganda ~189, UDHS 2022)

17. Partograph rules (WHO modified partograph, used in Uganda)

Item	How often	Rule
Start the partograph	When in active labour (cervix ≥4 cm on the modified partograph)	Plot the first VE on the alert line
Cervical dilatation	4-hourly VE (more often if indicated)	Plot as "X"
Descent (fifths palpable abdominally)	4-hourly (before each VE)	Plot as "O"
Fetal heart rate	Every 30 min in the first stage; every 5 min (after each contraction) in the second stage	Normal 110–160 bpm
Contractions	Every 30 min (count over 10 min)	Record frequency and duration (<20 s, 20–40 s, >40 s)
Liquor and moulding	With each VE	C clear, I intact, M meconium, B blood, A absent; moulding O, +, ++, +++
Maternal pulse	Every 30 min	
BP	4-hourly	
Temperature	2-hourly	
Urine (volume, protein, acetone)	Every void	

Line	Definition	Action if crossed
Alert line	Starts at 4 cm , slopes at 1 cm/hour	At HC II/III: refer to a facility with CS capability. At hospital: reassess (contractions, position, CPD, bladder), increase monitoring
Action line	Parallel, 4 hours to the right of the alert line	Full assessment by a clinician: exclude obstruction/CPD. If no CPD and membranes intact → ARM ; if contractions inadequate → oxytocin augmentation (not in a grand multipara without senior review, and never with obstruction). If CPD, obstruction or fetal distress → caesarean section

NOTE

WHO Labour Care Guide (2020), being introduced in Uganda, defines active labour from **5 cm**, drops the alert/action lines, and uses **time-at-each-centimetre alerts**: 5 cm ≥6 h, 6 cm ≥5 h, 7 cm ≥3 h, 8 cm ≥2.5 h, 9 cm ≥2 h; second stage ≥3 h (nulliparous) or ≥2 h (parous). Other alerts: FHR <110 or ≥160, late decelerations, pulse <60 or ≥120, SBP <80 or ≥140, DBP ≥90, temperature <35.0 or ≥37.5°C, contractions ≤2 or >5 per 10 min.

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